## APPENDIX 7 – NUM/MUM MATRIX

***For each question, please select only one answer that best describes the ward/unit/program/service that you are the manager of. Where more than one response may apply, please select the highest applicable.***

**Complexity – Description**

*Complexity relates to the levels of interaction of multiple measurable factors such as:*

*Bed/chairs/suites*

*Hours of service*

*FTE and proportion of part-time staff*

*Composition of services offered*

*Interfacing with multiple teams*

*Size of budget/business management responsibility*

*Throughput*

*Diversity of staff managed*

*Regional/isolated setting (away from hospital campus)*

| | **Criteria** | **Please select one from each column A-F** | | | | | | |
|---|---|---|---|---|---|---|---|---|
| | | **A** | **B** | **C** | **D** | **E** | **F** | **G** |
| 1 | Hours of Business | | The ward/unit/program/ service operates only during business hours up to 5 | | Operates 5-7 days a week (not nights) | Operates 5 - 7 days a week (not nights) plus on call service | More than 2 shifts per day, up to 6 days per week | The ward/unit /program/ service operates 24/7 – up to 365 days |

| | Criteria | Please select one from each column A-F | | | | | | |
|---|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F | G |
| | | | days a week (Mon to Fri) | | | | | |
| 2 | Nominated Birthing Suites | Not applicable | Up to 3 | 4 to 7 | 8 to 12 | 13 or more | | |
| 3 | Number of Perioperative services/Operating suites/ Theatres and/or Day Surgery suites includes CSSD services | Not applicable | Up to 2 | 2 | 3 to 4 | 5 to 7 | 8 to 10 | 11 or more |
| 4 | Post Anaesthesia Care Unit and holding bays | Not applicable | Up to 4 | | 5-8 | 9-14 | 15-20 | 21 or more |
| 5 | Bed based services (wards/unit) including the remaining component that has not already been included in questions 2, 3, 4 or 6 above (this includes HITH) | Not applicable | Up to 15 beds | | 16 to 24 beds | | 25 to 32 beds | 33 or more beds |
| 6 | Emergency Department treatment spaces (includes beds, trolleys, chairs etc) (including SSU, urgent care and any other clinical areas within this environment) | Not applicable | Less than 4 | 4 to 10 | 11 to 20 | 21-30 | 31 to 45 | 46 or more |
| 7 | Ambulatory/ /maternity assessment units/community services per day (e.g. Oncology, endoscopy, HARP, Dialysis, medical imaging, District Nursing Service, transit lounge, continence, chronic pain, wounds) *NOTE: Day Surgery at question 3 above* | Not applicable | Less than 7 treatments / patients/ clients | | 8 – 16 treatments/ patients/ clients | | 17-24 treatments/ patients/ clients | 25 or more |

| | Criteria | Please select one from each column A-F | | | | | | |
|---|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F | G |
| 8 | The number of different specialty clinics/community services/Ambulatory programs that you manage (*not how many sessions are actually run*) | Not Applicable | 1 | 2 | 3 | | 4 | 5 or more |
| 9 | The EFT within the area for which the NUM/MUM has responsibility (includes all staff of all disciplines who work in your ward / unit / program / service) that you have operational responsibility for. | | Less than 21 | 21-40 | 41-60 | 61-85 | 86-100 | 101 plus |
| 10 | Part-time head count as a proportion of total head count | | Part-time Headcount up to 20% | | Part-time Headcount 21 to 40% | | Part-time headcount 41 to 60% | Part-time 61% or more |
| 11 | Composition of services/mixed ward (*e.g. a regional or rural ward comprising medical and postnatal beds with or without responsibility to also provide care to ED / UCC presentations OR a sub-acute ward with GEM, rehabilitation beds and/or Transitional Care Program or medical ward with oncology and renal beds in the one ward*) and/or mixed age groups in the one ward/unit/program/service. For example, where more than one ratio applies. Please note this excludes ad hoc outliers. | | Single | | 2 | 3 | | 4 or more |

| | Criteria | Please select one from each column A-F | | | | | | |
|---|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F | G |
| 12 | Operational Budget (reference may be the current financial year) | | Up to $2 million | | $2 million but less than $4 million | | $4 - $6 million | More than $6 million |
| 13 | Are you the sole Midwife/Nurse Unit Manager at the campus in the absence of the DON? | | NO | | | | | YES |

| | Criteria | Please select one from each column A-F | | | | | |
|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F |
| 14 | Interface with others related to patient journey | Internal (with another department/ services/ward/ unit/program on the same hospital campus e.g. radiology) | Intra (across health service/ campuses) | A & B | External (outside the health service e.g. GP or Home Care Services) | A & D | Internal and external and intra (A, B and D) |
| 15 | Patient/resident/consumer/client (however titled) throughput in your work area | Same (or less than) throughput as past 12 months | | Increase up to 10% over the past financial year | | Greater than 10% over the previous financial year | |
| 16 | Annual throughput (Previous Financial year Episode of Care/separations) | Less than 300 | 301-600 | 601-1000 | 1001-3000 | 3001-6000 | 6001+ |

| | Criteria | Please select one from each column A-F | | | | | |
|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F |
| 17 | Enterprise Agreements | 1 Enterprise Agreement (e.g. all staff employed under the Public Nurses and Midwives EA) | | 2 EBA's e.g. staff employed under the Public Nurses and Midwives EA and one other EBA | 3 EBA's | | 4 EBA's or more |
| 18 | • People in learning positions over the past 12 months of any discipline:<br>• School based/work experience<br>• Undergraduates (VET/TAFE/Uni) on placement<br>• Techs and/or traineeships<br>• Graduate Program positions<br>• 'Refresher' program participants<br>• Post-Graduate students<br>• International bridging students<br>• Other Health Professional Students | NIL | 1 | 2 | 3 | 4 | 5 or more |
| 19 | Number of 'graduate program' positions and/or 'post-graduate positions'/training lines that are part of the ward/unit/program/ service roster | NIL | 2 or less | 3 to 5 | 6 to 8 | 9 to 11 | 12 or more |

**Level of Autonomy - Description**

*The Level of Autonomy is the degree to which the M/NUM manages and leads their unit. It may be affected by such things as the nature of the unit, the model of care and the involvement in Clinical Care, the presence or absence and access to the Director of Nursing and Midwifery and relevant support services. Taking into consideration any other operational reports that will influence this.*

| | **Criteria** | **Please select one from each column A-F** | | | | | |
|---|---|---|---|---|---|---|---|
| | | **A** | **B** | **C** | **D** | **E** | **F** |
| 1 | **Health Service Level (as defined in Safe Patient Care Act) that your ward/unit/program/service/program/service/service/ program is located** | Level 1 Hospital | Level 2 Hospital | Level 3 Hospital | Stand alone Aged Care Facility | | Level 4 Hospital |
| 2 | **Over the last financial year, what is the physical presence on campus of the DON/M on average?** | Full-time (5 days per week) | | More than 2 but less than 5 days per week | | Up to 2 days per week | Rarely |
| 3 | **Over the last financial year, what is the physical presence on campus of the CEO on average?** | Full-time (5 days per week) | | More than 2 but less than 5 days per week | | Up to 2 days per week | Rarely |
| 4 | **On the last financial year, what is the physical presence on campus/site of the immediate operational manager (if different to above)?** | Not applicable | Full-time (5 days per week) | More than 2 but less than 5 days per week | | Up to 2 days per week | Rarely |

| | Criteria | Please select one from each column A-F | | | | | |
|---|---|---|---|---|---|---|---|
| | | **A** | **B** | **C** | **D** | **E** | **F** |
| 5 | **How many managers in total, (including the M/NUM position being assessed) report to their Professional Line Manager? (This means your nurse/midwife senior e.g. ADON/M, DON/M, EDON/M).** | Up to and including 2 | 3 to 4 | | 5 to 6 | 7 to 8 | 9 or more |
| 6 | **How many people in total (including the M/NUM position being assessed report to their Operational Reporting Line?** *(may be a Program Manager, Site Manager, Divisional Director)* | Up to and including 2 | 3 to 4 | | 5 to 6 | 7 to 8 | 9 or more |
| 7 | **Out of hours/on-call roster responsibilities. For example, the position being assessed is identified/required as a contact for escalations such as staffing queries, clinical scenarios, requests for service provision and/or unplanned rostering shortfalls** | AHC or DON/M undertakes completely (Not required of M/NUM) | For your ward/unit/program/service only (not necessarily the campus) | Participating in an on-call roster for the campus at frequency of more than each month | Participating in an on-call roster for the campus at least once each month (afternoons, not nights) | Participating in an on-call roster for the campus at least once each month (nights included) | NUM/MUM responsible out of hours (*for the campus*) |

**Access to Health Service Infrastructure and Support – Description**

*This means as a leader and manager the MUM/NUM utilises, where available, additional expertise to perform their role*

| | **Criteria** | **Please select one from each column A-F** | | | | | |
|---|---|---|---|---|---|---|---|
| | | **A** | **B** | **C** | **D** | **E** | **F** |
| 1 | **Recruitment contributions:**<br>**i. PD review/ development**<br>**ii. advert**<br>**iii. review resumes**<br>**iv. interview**<br>**v. referee check**<br>**vi. appointment** | Involvement in one component e.g. interviews | Involved in 2 components | Involved in 3 components | Involved in 4 | Involved in 5 components | Involved in the entire process |
| 2 | **Based on the headcount, the number of staff Performance Development Review/Annual Appraisals which you complete as well as those which you delegate** | 20 or less | 21-40 | 41-60 | 61-100 | 101 to 200 | 201 plus |
| 3 | **Performance Improvement Plan *(for your staff)*** | Human Resources (HR) /Industrial Relations (IR) led always | | Shared process between M/NUM and HR/IR and/or another e.g. educator | | Predominately M/NUM led | M/NUM independently e.g. HR not directly involved *(may use* |

| | Criteria | Please select one from each column A-F | | | | | |
|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F |
| | | | | | | | *template supplied by HR)* |
| 4 | **OHS Responsibilities includes:**<br>**a) hazard identification**<br>**b) assessment of risks**<br>**c) controls**<br>**d) monitor and review**<br>**e) revise the control (if not working)** | | Within ward/unit/program/service the M/NUM leads the OHS processes with direct involvement of another (e.g. OHS/HR/ER) | | Within ward/unit/program/service the M/NUM leads the OHS processes with advice (e.g. from OHS/HR/ER) | | Within ward/unit/program/service the M/NUM manages the OHS processes including escalating notifiable incidents |
| 5 | **Return To Work (RTW).** *Please note this is to be assessed on what occurs rather than what may be the 'ideal' This considers WorkCover and Personal Injury/Illness* | M/NUM rarely involved as RTW Officer/OHS Staff/HR/ Employee Relations undertake entire process | | M/NUM is involved but led by others (e.g. RTW Officer/OHS Staff/HR/ Employee Relations) | | | M/NUM manages the RTW process with advice from another (e.g. RTW Officer/OHS Staff/HR/ Employee Relations) |
| 6 | **Projects (of any type)** *Please note this criteria is intended to be broad and not limited by 'size of project' or whether projects attract funding. Please select highest rating that* | Rarely involved as undertaken by others | Ward/unit/ program/service based with M/NUM available | Health Service Project with M/NUM available to be involved | Shared process | Ward/unit/program/service based project by M/NUM | Health Service Project managed by M/NUM |

| | Criteria | Please select one from each column A-F | | | | | |
|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F |
| | *has applied to the M/NUM position being assessed* | | to be involved but led by others | but managed by others | | | |
| 7 | **Business Intelligence and Clinical Analytics** For example, data for health services to use to inform strategy development, reform initiatives, and to support service development and capital planning | Not involved | Collect information | Collect and report | Collect, report and analyse | Collect, report, analyse and make recommendations | Collect, report, analyse, make recommendations and implement |
| 8 | **Involvement in co-ordinating your ward/unit/program/ service education/CPD events** | Little to no involvement of local education/CPD events | | Co-ordination is shared with one other e.g. Clinical Nurse Educator | Co-ordination is shared with more than one other and may include the M/NUM providing sessions | | Predominantly responsible for ward/unit/ program/service based education/CPD events and may include the M/NUM providing sessions |

***For each question, please select only one answer from Columns A through E that best describes the Ward/unit/program/service that you are the manager of.***

**Responsibility - Description**

*Responsibility refers to professional leadership for patient care and accountability for management of staffing and other resources within the ward/unit/program/service with a focus on improving patient outcomes.*

| | **Criteria** | **Please select one from each column A-E** | | | | | |
|---|---|---|---|---|---|---|---|
| | | **A** | **B** | **C** | **D** | **E** | **F** |
| 1 | **Business role / budget / capital expenditure** | M/NUM is not involved | Monitoring of budget that has been prepared by others e.g. finance | M/NUM drafts budget and monitors budget but budget finalised/approved by others | Business case development (may be ad hoc) | Revenue generation (e.g. completion of Aged Care Funding Instrument or resident recruitment) | Two or more of B, C, D & E |
| 2 | **Emergency responses** | Responding to local emergencies (own unit) | Responding to one other unit/ward in addition to A | A member of the site wide code team | Emergency controller/warden for site wide responses | Part of external emergency response team on behalf of hospital | |
| 3 | **Rostering** | Preparation of Roster is delegated to another but M/NUM retains responsibility to authorise/check compliance with | | M/NUM collaboratively undertakes both development and publishing of the roster with others | The M/NUM develops and publishes the roster | The M/NUM has responsibility for variations of staff in their ward/unit/program/ service | Develop, publish and variations |

| | Criteria | Please select one from each column A-E | | | | | |
|---|---|---|---|---|---|---|---|
| | | A | B | C | D | E | F |
| | | Safe Patient Care Act and skill mix | | | | | |
| 4 | **Staff replacement (unplanned absences)** | Centrally managed with limited input/time required from ward/unit/program/ service for this responsibility | A combination of central management and After Hours Co-ordinators/HR with limited input/time required from ward/unit/program/ service | You are responsible during business hours with the assistance of others (e.g. bank admin) i.e. NUM undertakes | Involved in arrangements for staff replacement within and out of hours some of the time i.e. NUM and ANUM or AHC seek replacement | Responsible for all arrangements for staff replacement within and out of hours i.e. NUM undertakes | |
| 5 | **Staff replacement (planned absences e.g. Annual Leave or Long Service Leave)** | Centrally managed with limited input/time required from ward/unit/program/ service M/NUM for this responsibility | A combination of central management and After Hours Co-ordinators /HR with limited input/time required from M/NUM | You are responsible during business with the assistance of others (e.g. bank admin) | Involved in arrangements for staff replacement in and out of hours some of the time | Responsible for all arrangements for staff replacement in and out of hours | |
| 6 | **Equipment/ Products** | Another person sources new equipment/product s for the ward/unit/program/ service | Participate in trial/testing of products/equipmen t | Makes recommendations to another in relation to new equipment/product s for the ward/unit/program/ service (may include provision of quotes) | Responsible for sourcing and keeping up to date with new equipment/product s for the ward/unit/program/ service | Health Purchasing Victoria participant (as representative of your Health Service) | Contract management responsibilities for the ward/unit/program/ service in relation to equipment/ products |

**Governance Obligations – Description**

*Governance Obligations refers to actions taken by leaders at all levels and systems, established for improving the quality of services and safe-guarding high standards of care, by creating an environment in which excellence in clinical care will flourish. This may include involvement in accreditation processes.*

| | **Criteria** | **Please select one from each column A-E** | | | | | |
|---|---|---|---|---|---|---|---|
| | | **A** | **B** | **C** | **D** | **E** | **F** |
| 1 | **Board/or Board sub-committees** | No interaction | Preparing written briefings, but not attending meetings | Preparing presentation, but not attending meetings | Participation at meetings (may be ad hoc) | Member of the Board/Sub-committee meetings | |
| 2 | **Executive** | No interaction | Preparing briefings, but not attending | Preparing presentation, but not attending meetings | Participation at Executive meetings (may be ad hoc) | Member of the Health Service Executive | |
| 3 | **Program Involvement** | Not applicable | Preparing briefings, but not attending | Preparing presentation, but not attending meetings | Participation at Program meetings (may be ad hoc) | Member of the Program committee | |
| 4 | **Committee involvement** | Implementing actions arising from health service/ campus meetings/ committees | Conducting unit based meetings (local) e.g. ward meeting | Participation/ attendance at Health Service/campus meetings | Chairing Health Service/campus meetings from time to time (may involve preparation of agenda and/or minutes) | Collate summation from committee for the Board and/or Executive | A-E |

| | | | | | | | |
|---|---|---|---|---|---|---|---|
| 5 | **External stakeholders, for example but not limited to professional associations, businesses, Primary Health Network related, NGOs, educational organisations, another funding organisation, consumer participation or other** | No participation | Communication with external stakeholders | Participating in forums (can include professional, operational and other) | Being the health service nominee for the engagement with external stakeholders e.g. Police/AV/DHHS/NDIS/ SCV/Diabetes Australia/Justice | Reporting outcomes to the Executive/Program Director as a result of your engagement with external stakeholders e.g. Police/AV/DHHS/NDIA/ SCV/Diabetes Australia/Justice | |
| 6 | **Policy and procedures** | Monitoring and oversight of staff applying within your ward/unit/program/service | Input into development/ review (may be ad hoc) | Lead the writing, and/or review for your ward/unit/ program/service (may be ad hoc) | Lead the writing, and/or review for Health Service P&P (including campuses where applicable). May be ad hoc | Responsible for development and implementation of Health Service P&P. May be ad hoc | A-E |
| 7 | **Clinical incidents/clinical risk management (e.g. VHIMS)** | Within your ward/unit/ program/ service, monitoring and responding to incident reporting | Contributing to review and escalating risk reports | Undertaking investigation and reporting or implementing actions arising | Participation in full case review and/or internal root cause analysis | Lead full case review and/or internal root cause analysis | Provide assistance/ input to other Health Services RCA |
| 8 | **Complaints and feedback (both** | Process is managed entirely by others | | The process is shared e.g. with the | Process involves multiple | The M/NUM manages the entire process and may include investigation | The M/NUM will independently |

| | | | | | | | |
|---|---|---|---|---|---|---|---|
| | **formal and informal)** | | | Complaints Liaison Officer | departmental consultations | and drafting response(s) | manage the entire process |
| 9 | **Quality Improvement (QI)** | QI planning is undertaken by another within the organisation | Contributes to QI planning for your ward/unit/ program/service | Monitor and report on QI activities (which have been planned by others) | Lead QI activities within the ward/unit/program/ service which may include providing reports to others on same | Lead QI activities within the Health Service which may include providing reports to others on same | |
| 10 | **Operational/ business plan** | Planning is undertaken by another within the organisation | Contribute to the development of the operational/ business plan for your ward/unit/service/program | | Lead the development of the operational/business plan in your ward/unit/service/program | Responsible for writing the operational/business plan for the ward/unit/program/service yourself | Responsible for writing the operational/ business plan for the Health Service yourself |